BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Full blood count, kidney and liver tests, blood pressure, thyroid tests and assessment of heart function.	Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check international normalised ratio (blood-clotting test) (INR) as indicated if taking warfarin.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself.	Keep a current dose and symptom record.	Tell the team before surgery or dental procedures. Report painful palms or soles early; skin care and early dose review can help prevent severe hand-foot reactions.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	Thyroid tests monitor hormone balance. Thyroglobulin and other response tests depend on the cancer subtype and specialist plan; they are not substitutes for scans.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00294 v4: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, blood pressure, thyroid tests and assessment of heart function.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

Thyroid tests monitor hormone balance. Thyroglobulin and other response tests depend on the cancer subtype and specialist plan; they are not substitutes for scans.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Sorafenib: differentiated thyroid cancer: treatment and checks

Progressive, locally advanced or metastatic differentiated thyroid cancer that no longer responds to radioactive iodine.

Sorafenib 400 mg twice daily, every day, until progression or unacceptable side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, blood pressure, thyroid tests and assessment of heart function.	Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself.	Keep a current dose and symptom record.	Tell the team before surgery or dental procedures. Report painful palms or soles early; skin care and early dose review can help prevent severe hand-foot reactions.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	Thyroid tests monitor hormone balance. Thyroglobulin and other response tests depend on the cancer subtype and specialist plan; they are not substitutes for scans.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

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Treatment and review

Twice daily, continuously

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.50